INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Nivolumab · every 2 weeks · previously treated non-small-cell lung cancer (NSCLC)

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
14 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00483
Version 13a

One treatment visit every 2 weeks; your team confirms duration.

Treatment stages

01 **Nivolumab monotherapy**
Repeat treatment and review

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCHECK · CHAPTER 01

Understanding your treatment

Nivolumab · every 2 weeks · previously treated non-small-cell lung cancer (NSCLC) · National Cancer Control Programme (NCCP) 00483

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact the oncology team immediately for new or worsening breathlessness, chest pain, persistent diarrhoea, severe tummy pain, yellow skin/eyes, much less urine, severe headache, marked weakness, confusion, unusual thirst or passing much more urine, or a spreading/blistering rash. For severe breathing difficulty, collapse, seizure or other immediately dangerous symptoms, seek emergency care. Do not wait for the next infusion.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

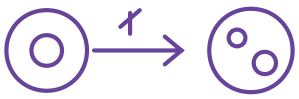
How nivolumab works

Aim to control advanced lung cancer, relieve symptoms and prolong life where possible.

IMMUNOTHERAPY

Nivolumab

Opdivo



Blocks PD-1 on immune cells

Nivolumab blocks PD-1 on immune cells, helping them recognise and attack cancer. It can also cause inflammation in healthy organs.

Immune cell → PD-1 blocked → Cancer attacked

What to watch for

Report new breathing, bowel, skin, hormone, heart or nerve symptoms promptly; these may be immune inflammation.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your cancer and treatment aim

Previously treated locally advanced or metastatic non-small-cell lung cancer (NSCLC).

Eligibility

Adults with locally advanced or metastatic NSCLC after prior chemotherapy; recurrence or progression during or after one prior platinum-containing doublet for advanced/metastatic disease; ECOG 0–1.

Choices

Discuss other treatment, trials and symptom-focused care with your team.

How nivolumab works

Immunotherapy supports immune attack.

PD-1 blockade

Nivolumab blocks PD-1 on immune cells, helping them recognise and attack cancer. It can also cause the immune system to inflame healthy organs.

Healthy organs

Inflammation can affect lungs, bowel, liver, kidneys, skin, glands, muscles, heart and nerves. Some effects are serious, delayed or permanent.

Why this treatment may be offered

Nivolumab can help the immune system control some previously treated lung cancers. Some people benefit for a long time; others do not benefit.

Trial results describe groups, not an individual prognosis.

SACTCHECK · CHAPTER 02

Your schedule, tests and appointments

Nivolumab · every 2 weeks · previously treated NSCLC · NCCP 00483

Your treatment and reviews

Nivolumab · every 2 weeks · previously treated NSCLC

14-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

REPEAT TREATMENT AND REVIEW

Nivolumab monotherapy

Cycle 1 | Cycle 2 | Cycle 3 | …

Nivolumab 240 mg intravenous (IV) over 30 minutes OR 600 mg as a subcutaneous injection over 3–5 minutes on Day 1 every 2 weeks. Your team confirms one formulation; the whole visit includes checks and preparation. Continue until progression or unacceptable toxicity, with individual benefit and safety review.

There is no planned chemotherapy induction or maintenance switch in this monotherapy pathway. Treatment is reviewed regularly and may be held, stopped or changed.

BASELINE → TREATMENT → FOLLOW-UP

Checks across your treatment

This is a relative treatment map, not a booking calendar. Ask who books each test and reviews the result. Extra tests may be needed for symptoms.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00483 v13a · official regimen

THE WHOLE TREATMENT JOURNEY

Nivolumab · every 2 weeks · previously treated NSCLC: treatment and checks

Nivolumab 240 mg IV over 30 minutes OR 600 mg as a subcutaneous injection over 3–5 minutes on Day 1 every 2 weeks. Your team confirms one formulation; the whole visit includes checks and preparation. Continue until progression or unacceptable toxicity, with individual benefit and safety review.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First treatment	Between treatments	Response review	After treatment
Blood count, kidneys, liver and glucose NCCP 00483 · SMPC	Baseline tests	Results checked before treatment	Before every 2-week cycle	Continue during holds as directed	Follow unresolved abnormalities
Thyroid and hormones NCCP 00483 · SMPC	Baseline thyroid	Review symptoms/ medicines	Thyroid once a month; cortisol if indicated	Replacement/ steroid plan reviewed	Continue endocrine care when needed
Symptoms and immune effects NCCP 00483 · SMPC	Record existing symptoms	Report reactions immediately	Report new symptoms any day	Clinical hold/ restart review	Monitor at least 5 months after last dose
Cancer response NCCP 00483 · SMPC	Confirm previous platinum treatment	Agree review plan	Scans as consultant directs	Progression review before cycle 8	Agree follow-up/next treatment

What each check is for

1 **Eligibility**

Before first dose

Verify source indication

Ask / act: Confirm prescription, consent, baseline tests and alert card.

2

Response review

Before cycle 8

Assess progression

Ask / act: Document the response and continuation decision.

3 **After final dose**

At least 5 months

Immune effects can be delayed

Ask / act: Provide ongoing symptom/urgent contact and hormone-care plan.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Appointments

STEP 2

Course length

STEP 3

Tests and review

Blood count, kidney/liver tests and glucose before every 2-week cycle; thyroid once a month; cortisol if clinically indicated. Review cancer progression before cycle 8. Continue immune-toxicity monitoring for at least 5 months after the last dose.

STEP 4

Home records

Carry your immune alert card, medicine list and urgent contact details. Follow prescribed steroids or hormone replacement carefully; do not stop steroids suddenly.

STEP 5

After treatment

Keep reporting symptoms and attend agreed follow-up for at least 5 months after the final dose; some hormone replacement continues longer.

Your treatment course

Confirm the formulation with your team.

Pauses

Treatment may be held or stopped for side effects. Nivolumab is not routinely dose-reduced. Tell the team about missed appointments.

Tests and review

New symptoms matter even when recent tests were normal.

Before starting

Blood count, kidney/liver tests, glucose and thyroid tests establish a baseline; discuss existing illness and all medicines.

Response review

Your team assesses progression before cycle 8; later scans follow the consultant plan.

SACTCHECK · CHAPTER 03

Symptoms and supportive care

Nivolumab · every 2 weeks · previously treated NSCLC · NCCP 00483

Day 1 → Day 14: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Treatment	 **Day 1** Day 1 · prescribed infusion or injection
Immune effects	 **Days 1–14** Any day · also after treatment ends
Energy, skin and bowels	 **Days 1–14** Timing varies · report changes promptly

Using the map: A relative treatment map, not a booking calendar.

Symptoms: Immune effects can begin during treatment or after it stops.

Treatment duration: Continue until progression or unacceptable toxicity, with individual benefit and safety review.

> **Why these bands stay broad:** Days show a typical interval between treatments; appointments can change after clinical review. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment.

> **There are no guaranteed "safe days".** Immune effects can happen on any day and after treatment ends. Carry your immunotherapy alert card and tell any healthcare team that you receive nivolumab.

Between-treatment notes

Day 1: Your treatment visit

The team checks blood results, symptoms and medicines before the prescribed infusion or injection. Tell the nurse immediately about itching, flushing, breathlessness, dizziness or feeling suddenly unwell. Before leaving, confirm the next tests, treatment visit and urgent contact number.

Day 2–14: Between visits

You may feel well between infusions, but immune inflammation can begin at any time. Keep a short symptom record and take medicines exactly as prescribed. Contact your team about new symptoms instead of waiting for your next appointment. Confirm monthly thyroid tests and who arranges them; report new symptoms without waiting for the next treatment.

Support around the medicines

1 

Sickness medicines

Nivolumab alone has minimal sickness risk. Routine preventive anti-sickness medicines or premedication are not usually required; the team may prescribe symptom relief.

2

Immune alert card

Carry your alert card during treatment and afterwards. Show it when seeking urgent care; symptoms can be mistaken for an infection or an ordinary stomach upset.

3

Hormones and steroids

Some immune effects need steroids or hormone replacement. Follow the prescribed plan and do not stop steroids suddenly. Ask for help if symptoms worsen or you cannot take essential medicines. Hormone replacement alone does not always mean nivolumab must stop. Your team checks symptoms and recovery and reviews any medicines that suppress the immune system before deciding whether treatment can continue or restart.

4

Pregnancy and breastfeeding

Discuss pregnancy, fertility and breastfeeding before treatment. Use effective contraception during treatment and for at least 5 months after the last dose. Breastfeeding requires a specialist decision; contact your team promptly if pregnancy occurs.

Practical support

Tell your team what matters to you.

Energy and activity

Balance activity and rest. New marked fatigue with dizziness, headache, thirst or weakness needs assessment.



Medicines and pregnancy

Bring all medicine/supplement details. Discuss fertility and breastfeeding. Use effective contraception during treatment and for at least 5 months after the final dose.

Sickness

Minimal sickness risk: routine preventive sickness medicines are not usually required. Your team may prescribe symptom relief.

When to get help

Use your oncology urgent contact plan.

Report promptly

Report new breathlessness, persistent diarrhoea, severe pain, yellow eyes, reduced urine, severe headache, weakness, unusual thirst or a spreading/blistering rash immediately. Use your oncology contact plan; severe breathing difficulty, collapse, seizure or severe chest pain needs emergency care.

During treatment

Tell the nurse immediately about a reaction during the infusion or injection.

After treatment

Continue monitoring for at least 5 months after the final dose. Show your immune alert card to anyone treating you.

Questions worth asking

Why am I having nivolumab?

Previously treated locally advanced or metastatic non-small-cell lung cancer (NSCLC). It is used after previous chemotherapy to help control cancer; ask how the benefits and alternatives apply to you.

What are my other options?

Ask about other anticancer treatment, trials and symptom-focused care suited to your cancer and preferences. Support remains available if you decline treatment.

Try explaining the plan back: Which medicines are you having now? What changes after induction? What would make you call today? Ask your team to fill any gaps.

Previously treated NSCLC Previously treated locally advanced or metastatic non-small-cell lung cancer (NSCLC). This patient suite is lung-focused; other shared protocol indications need their own counselling.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Nivolumab every 2 weeks

Active medicines: nivolumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Minimal emetic risk; no routine antiemetic prophylaxis or premedication. Prescribe rescue/support individually.
- Immune alert card and immediate symptom reporting.
- Blood count, kidney/liver tests and glucose before every 2-week cycle; thyroid once a month; cortisol if clinically indicated. Review cancer progression before cycle 8. Continue immune-toxicity monitoring for at least 5 months after the last dose.
- Effective contraception during treatment and at least 5 months afterwards; specialist breastfeeding discussion.
- Current NCIS medical-oncology guidance does not create a routine cytotoxic prescription for minimal-risk nivolumab monotherapy.
- Hormone replacement alone does not always mean nivolumab must stop. Your team checks symptoms and recovery and reviews any medicines that suppress the immune system before deciding whether treatment can continue or restart.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Hormones / nervous system

Nivolumab

Severe headache, confusion, weakness, dizziness or marked thirst need prompt advice.

3 Heart

Nivolumab

Chest pain, racing heartbeat or fainting needs urgent assessment.

5 Kidneys

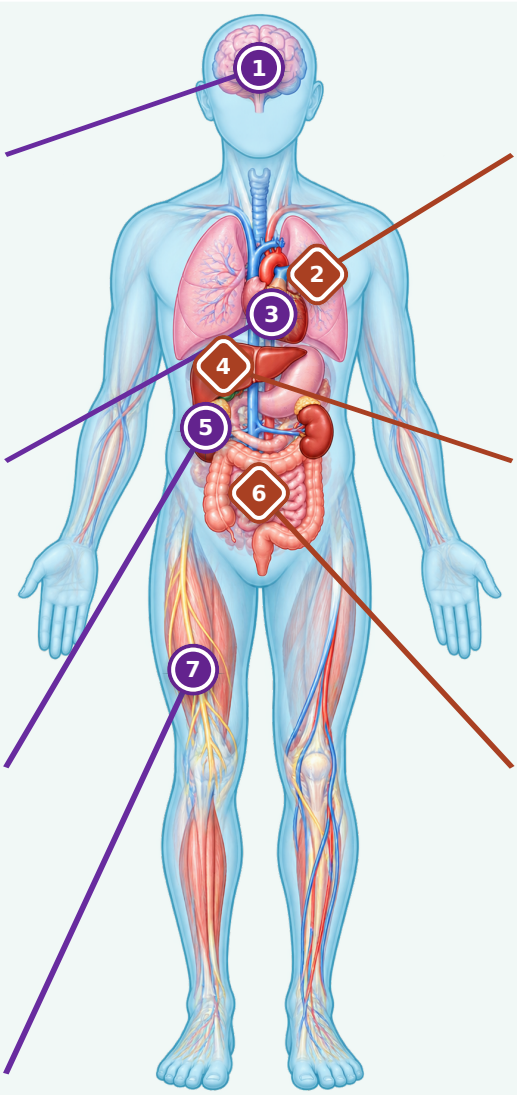
Nivolumab

Report much less urine, new swelling or a change in kidney tests.

7 Skin / muscles

Nivolumab

Spreading rash, blisters, severe weakness or muscle pain needs prompt review.



2 Lungs

Nivolumab

New cough or breathlessness can signal inflammation or another urgent cause.

4 Liver

Nivolumab

Yellow eyes/skin or dark urine may signal inflammation; tests also check the liver.

6 Bowel

Nivolumab

New or persistent diarrhoea, blood in stools or severe tummy pain needs urgent advice.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact the oncology team immediately for new or worsening breathlessness, chest pain, persistent diarrhoea, severe tummy pain, yellow skin/eyes, much less urine, severe headache, marked weakness, confusion, unusual thirst or passing much more urine, or a spreading/blistering rash. For severe breathing difficulty, collapse, seizure or other immediately dangerous symptoms, seek emergency care. Do not wait for the next infusion.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

SACTCHECK · CHAPTER 04

Sources and further information

Nivolumab · every 2 weeks · previously treated non-small-cell lung cancer (NSCLC) · National Cancer Control Programme (NCCP) 00483

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00483 v13a · official regimen**
   https://healthservice.hse.ie/documents/6498/483_Nivolumab_14-day_.pdf
2. **EMA Opdivo current product information**
   https://www.ema.europa.eu/en/documents/product-information/opdivo-epar-product-information_en.pdf
3. **CheckMate 017/057 five-year pooled follow-up, Borghaei 2021**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC8078445/
4. **HSE · chemotherapy safety and when to contact your team**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
5. **NCCP NCIS medical-oncology antiemetic guidance v8,2025**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf